

MARKETING AND PUBLICATION MATERIALS

This document contains the marketing and publication materials for Transforming American Healthcare: A Six-Pillar Framework for System Transformation. These materials are intended for use with publishers, distributors, media, and conference organizers.

Back Cover Copy

TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation

American healthcare is not failing randomly. It is failing systematically — along the precise fault lines that everyone can see but that the system's financial architecture makes almost impossible to change. Hospital prices that are three to four times Medicare rates. Insurance premiums that have doubled in six years while wages grew a fraction as fast. A clinical workforce buried in administrative burden. A rural hospital network designed for a population that no longer exists. And a payment system that rewards volume over value, sickness over health, and institutional complexity over community need.

This book is about what it looks like when a state actually tries to change all of it at once.

Vermont — with 647,000 people, 14 hospitals, and a healthcare crisis that has arrived earlier and more severely than anywhere else in the country — has undertaken the most comprehensive state-level healthcare transformation in American history. Its legislature has mandated reference-based pricing for hospitals. It has committed to mandatory global hospital budgets. It has signed onto the federal AHEAD Model, making Vermont and five other states accountable for total cost of care across all payers. It has invested \$195 million in rural health infrastructure. And it has given its Agency of Human Services a statutory mandate to produce — by December 2028 — a Statewide Health Care Delivery Strategic Plan that commits the system to specific, measurable outcomes for affordability, quality, equity, and access.

Vermont is not a model of success. It is a model of seriousness. And in American health policy, seriousness is rare enough to be worth studying.

Transforming American Healthcare develops a six-pillar analytical framework — Policy, Economics, Technology, Clinical, Equity, and Operations — and applies it to Vermont's transformation with the full weight of primary source documentation: Acts 167 and 68, the Oliver Wyman hospital financial analysis, the GMCB's reference-based pricing data, the AHEAD State Agreement, and the Vermont Rural Health Transformation Program application. The result is simultaneously a rigorous academic treatment of healthcare transformation theory and a practitioner's field guide for the healthcare leaders, policymakers, and analysts who must execute transformation in the real world.

Vermont is not just Vermont. It is every state, ten to twenty years from now. The forces producing Vermont's crisis — aging demographics, commercial cross-subsidy collapse, voluntary reform failure — are already present nationally. Vermont is generating, in

public, the evidence base for the questions every state will eventually need to answer. This book documents that experiment while it is still running, and gives readers the analytical framework to understand what it means.

Health Transformation Review

htreview.com

About Health Transformation Review

Health Transformation Review (HTR) is the leading intelligence and advisory platform for healthcare transformation professionals. Through the HTR Intelligence Feed, Research Lab, Academy, and Advisory services, HTR provides the analytical frameworks, operational tools, and strategic guidance that healthcare organizations, state agencies, and policymakers need to navigate — and lead — the transformation of American healthcare.

HTR's work is organized around the six-pillar framework developed in this book: Policy, Economics, Technology, Clinical, Equity, and Operations. The Research Lab's analytical tools — including the APM Shared Savings Calculator, VBC Transformation Readiness Assessment, Health Equity Studio (HEROI), Hospital Financial Stress Test, and AI Clinical Governance Checklist — implement the frameworks in this book for organizational-specific analysis.

HTR Advisory engagements serve hospitals, health systems, payers, state agencies, and policymakers on the specific transformation decisions that determine whether healthcare reform produces the results it promises: APM contract evaluation, transformation readiness assessment, health equity program design, technology architecture review, and transformation management.

HTR has worked extensively with Vermont healthcare organizations and government agencies throughout the transformation described in this book. The Vermont case study reflects both the extensive public record of Vermont's transformation and HTR's direct engagement with its execution.

For information about HTR subscriptions, tools, and advisory services: htreview.com | info@htreview.com

One-Page Executive Summary

TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation — Health Transformation Review, April 2026

The Problem

American healthcare is financially unsustainable in ways that incremental reform cannot fix. Hospital prices averaging 250-400% of Medicare rates drive insurance premiums that have doubled in six years. Nine of Vermont's fourteen hospitals reported operating losses in 2023; thirteen are projected to by 2028. Voluntary payment reform has not achieved the structural change required. The demographic mathematics of an aging population overwhelming a commercial cross-subsidy model are inescapable and worsening.

The Framework

This book develops a six-pillar analytical framework for healthcare transformation — Policy, Economics, Technology, Clinical, Equity, and Operations — grounded in the insight that transformation fails when any single pillar is addressed in isolation. Payment reform without clinical redesign produces financial pressure without behavioral change. Clinical models without data infrastructure cannot be managed at scale. Technology without governance produces tools no one uses effectively. All six pillars must move together.

The Vermont Case Study

Vermont is the book's primary teaching case — not because it is large or powerful, but because it is early. Early in demographic aging, early in hospital financial fragility, early in the recognition that voluntary reform cannot produce structural change, and early in the willingness to legislate structural responses. Acts 167 and 68, the AHEAD Model State Agreement, and the \$195M Rural Health Transformation Program award constitute the most comprehensive state-level transformation mandate in American health policy history. Vermont's outcomes will determine the national policy toolkit for the decade to come.

Key Findings

- Vermont's hospital system faces a \$700M-\$2.4B five-year cumulative deficit without transformation, with 13/14 hospitals projecting losses by 2028.
- Reference-based pricing (mandatory FY2027) and hospital global budgets (mandatory FY2028-2030) are the economic pillar's foundational mechanisms for breaking the commercial price-to-premium transmission chain.
- Vermont's Blueprint for Health produces a documented 5.8:1 ROI — \$5.8M reduction in medical expenditure per \$1M invested — the strongest evidence base for primary care investment of any state program in the country.
- The H.R. 1 Medicaid cuts (\$911B nationally, \$137B in rural areas over 10 years) are front-loaded with the \$50B Rural Health Transformation Program but backloaded with permanent Medicaid reductions — creating a fiscal cliff after 2030 that only structural transformation can prevent.
- Vermont's AHS restructuring mandate — to deliver a Statewide Health Care Delivery Strategic Plan by December 2028 — is the organizational design challenge at the heart of the transformation, requiring AHS to become a population health system operator rather than a program administrator.

For Which Readers

Healthcare executives and administrators navigating VBC contracts, capital decisions, and transformation planning. State policymakers designing or evaluating payment reform legislation. Health policy professionals tracking the federal-state policy landscape. Clinical and quality leaders redesigning care delivery for value-based

environments. Healthcare students and researchers building transformation analytical capability.

What Readers Will Take Away

A six-pillar analytical framework applicable to any healthcare system. The complete Vermont transformation playbook grounded in primary source documentation. Practical tools — APM financial modeling, VBC readiness assessment, HEDIS equity measurement, AI governance — for organizational application. The national forecasting context that positions Vermont's experiment in the landscape of American healthcare's next decade.

Advance Praise — Placeholder Template

The following are placeholder formats for advance praise to be solicited from Vermont healthcare leaders, national health policy experts, and healthcare transformation practitioners. Suggested targets and talking points are provided for outreach.

Category 1: Vermont State Government and Healthcare Leaders

"[Advance praise to be solicited] Vermont's transformation agenda requires the analytical rigor and operational depth that this book provides. It is the most comprehensive treatment of our healthcare reform work that has been written, and it is the reference that AHS staff, hospital leaders, and legislative partners should have in hand as we execute the Statewide Strategic Plan."

— Vermont Secretary of Human Services [name to be confirmed]

"[Advance praise to be solicited] The Green Mountain Care Board's work on reference-based pricing and global budgets required exactly the kind of rigorous analytical framework this book develops. Healthcare leaders in Vermont and beyond will find the six-pillar approach both intellectually compelling and practically actionable."

— Green Mountain Care Board Chair [name to be confirmed]

"[Advance praise to be solicited] Vermont's fourteen hospitals are navigating the most significant transformation of our operating environment in decades. This book gives our boards, executive teams, and clinical leaders the framework to understand what we are being asked to do, and why the transformation — difficult as it is — is the right path forward."

— Vermont hospital CEO [name to be confirmed]

Category 2: National Health Policy Voices

"[Advance praise to be solicited] Vermont is the most important healthcare policy experiment in the country right now, and this book is the definitive account of it. The six-pillar framework is the most analytically coherent structure for understanding healthcare transformation that I have encountered."

— National health policy researcher or CMMI official [name to be confirmed]

"[Advance praise to be solicited] The research on rural health equity in northern New England has produced powerful evidence about the structural forces driving rural health disparities. This book translates that evidence into a transformation framework that rural communities can actually use."

— Health Affairs / rural health researcher [name to be confirmed]

Category 3: Healthcare Executive Practitioners

"[Advance praise to be solicited] Every CFO and CEO in a value-based care environment should read Chapter 7 before signing their next APM contract. The financial modeling framework is exactly what the field has been missing: rigorous, practical, and Vermont-grounded."

— Health system CFO [name to be confirmed]

"[Advance praise to be solicited] The Technology pillar chapters are the most comprehensive treatment of FHIR implementation and AI governance in healthcare that I have seen in a single volume. Vermont's five-layer architecture is directly applicable to any state or health system building toward all-payer data management."

— Health IT executive [name to be confirmed]

Outreach Guidance

When soliciting advance praise, provide reviewers with: (1) The complete manuscript or the specific chapters most relevant to their expertise; (2) A suggested focus — e.g., 'We would particularly value your perspective on the Vermont equity data in Chapter 12'; (3) A four-week timeline for response; (4) The proposed attribution format (title, organization, and any desired caveats about speaking in personal vs. organizational capacity).

Priority outreach targets: Vermont AHS Secretary; GMCB Chair; at least two Vermont hospital CEOs; one CMMI or CMS official familiar with AHEAD; one KFF or national health policy researcher; one rural health equity researcher; two health system CFOs or CMOs with VBC experience.

Conference and Media Pitch

Elevator Pitch (30 seconds)

Vermont's hospital system is five years from financial collapse without structural reform. Its legislature has mandated reference-based pricing, global hospital budgets, and a Statewide Strategic Plan. This book uses Vermont as the teaching case for a six-pillar framework — Policy, Economics, Technology, Clinical, Equity, and Operations — that explains why healthcare transformation fails when any single pillar is missing, and what it looks like when all six move together. Vermont is not just Vermont. It is every state, ten years from now.

Conference Session Proposal

Session title: Vermont as National Template: What a Six-Pillar Healthcare Transformation Looks Like in Practice

Format: 60-minute presentation with 15-minute Q&A

Target conferences: AHIP, AHA Annual Conference, HIMSS, AMGA Annual Conference, NASHP State Leaders Forum, Academy Health Annual Research Meeting, State Innovation Exchange Health Policy Summit

Session abstract: Vermont's healthcare transformation — mandatory reference-based pricing, all-payer global budgets, the AHEAD Model, and a \$195M rural health capital investment — constitutes the most comprehensive state-level transformation attempt in American health policy history. This session uses the HTR six-pillar framework to explain why Vermont's approach differs from failed voluntary reform efforts, what the federal Medicaid cuts in H.R. 1 mean for states attempting structural reform, and what early evidence from Vermont's transformation suggests about whether the approach is working. Attendees will leave with a transferable analytical framework for evaluating transformation initiatives in their own organizations and states.

Media Talking Points

- *'Vermont is charging a health insurer \$200 for a procedure that Medicare pays \$100 for. Act 68 says that's illegal starting in 2027. That's what reference-based pricing means in practice.'*
- *'The \$50 billion Rural Health Transformation Program is the largest rural healthcare investment in American history. Vermont got \$195 million. But it expires in 2030, right when the Medicaid cuts get severe. That's the fiscal cliff every rural state needs to plan for.'*
- *'Vermont's Blueprint for Health produces \$5.80 in medical cost reduction for every \$1 invested in primary care. That is the clearest ROI number in American healthcare policy. Why isn't every state doing this?'*
- *'The GLP-1 drugs work. They reduce obesity, diabetes, and cardiovascular disease. But if you're on Medicaid in most states, you can't get them because of cost. That's not a medical decision — it's a policy decision. Vermont needs to make the right one.'*
- *'Vermont's Agency of Human Services has a statutory deadline — December 2028 — to deliver a plan for its entire healthcare system. No American state agency has ever been asked to do this on this timeline with this level of specificity. Whether they pull it off will tell us a lot about what state government can actually do.'*